Psych Emergency - Mental Condition

12YOF, suicide attempt by acetaminophen overdose

Scenario Set Up	Equipment: BSI/PPE, BP cuff, pulse oximeter, glucometer, stethoscope, gurney, radio, O2, BVM, AED. PROCTOR: You are a 12-year-old female sitting on your bedroom floor crying after intentionally swallowing an unknown number of pills from your family's medicine cabinet. Your mother found you with an open medication bottle and called 911. You have a history of depression, anxiety, and suicidal thoughts but no previous attempts. You have recently been having problems with friends at school and have become more withdrawn. You are nauseous, dizzy, embarrassed, and afraid of getting in trouble. You repeatedly say you are fine and do not want to go to the hospital. - IF EMTs approach calmly and reassure you, become more cooperative. - IF parents remain present, deny taking medication. - IF EMTs speak to you privately, admit to swallowing pills because you wanted to die. - IF asked about current suicidal thoughts, say, "I don't know, I'm scared now."
Dispatch	Respond Code 3 to a 12YOF following a possible intentional medication ingestion. Mother reports finding daughter crying with an open medication bottle nearby. Pt conscious and breathing, complaining of dizziness. Possible suicide attempt.
Scene Size Up	Scene safe, no immediate environmental hazards. E: - Pt sitting on bedroom floor crying, not aggressive. - Open medication bottle near pt. - Mother present and extremely distressed. - No obvious trauma or active bleeding. - Potentially life-threatening overdose despite stable appearance.

	N: 1 A: M: ingestion of medication E: simple S: none
Pertinent Primary Assessment Findings	- AVPU: A (Alert) - A&O x4 (person, place, time, event), GCS 15. Tearful but responds appropriately. A - Airway patent, speaking in full sentences through sobs. B - RR 24, slightly labored due to crying, otherwise regular. SpO2 96% on room air. C - Skin pink, warm, slightly diaphoretic. HR 118, radial pulse strong and regular, cap refill <2 sec.
Pertinent Secondary Assessment Findings	S - Nausea and dizziness, no vomiting. If parents present: "I'm fine, I didn't do anything." If interviewed privately: admits to intentional ingestion and fear of dying. A - none M - No prescribed medications. Ingested unknown number of acetaminophen 500 mg tablets (identified by mother). P - Anxiety, major depressive disorder, history of suicidal ideation. No previous suicide attempts. Mother reports increased withdrawal over recent weeks. L - Dinner approximately 4 hours ago. Normal food and fluid intake. E - Pt returned from school, completed homework, ate dinner, and went to bed. Mother later heard crying and found an open medication bottle. Ingestion approximately 30 minutes before EMS arrival. Additional Assessment: - PERRL, no neurological deficits. - Denies alcohol or recreational drug use. - Admits suicidal intent if interviewed privately. - When asked about current intent: "I don't know, I'm scared now." - No external trauma observed.
Vitals	Initial: BP: 108/68, HR: 118, RR: 24, BGL: 96, SpO2: 96% RA, T: 98.4°F After reassurance/in ambulance:

	BP: 104/66, HR: 110, RR: 20, BGL: 94, SpO2: 99% RA, T: 98.4°F
Treatments	1. O2 if needed, → anxiety is present and causing issues a. This can be a NC @ 2LPM 2. Transport
Key Points	Pediatric suicide attempts are medical emergencies requiring immediate evaluation and transport. - Acetaminophen overdose may initially present with minimal symptoms but can cause severe delayed liver injury. - Establish trust through calm, nonjudgmental communication. - Interview pt privately when appropriate to obtain accurate information. - Ask directly about suicidal thoughts and intent. - Identify medication, dose, and time of ingestion without delaying transport. - Do not assume pt is safe because she regrets the attempt. - Maintain continuous observation and follow local pediatric consent protocols. - Medical stabilization takes priority, followed by psychiatric evaluation.
Bonus Questions	**1. Why should EMTs interview the pt without her parents present?** A: Pt may fear punishment and withhold information. Privacy helps establish trust and obtain accurate history. **2. Can EMTs allow the pt to refuse transport if she says she no longer wants to die?** A: No independent refusal by the 12-year-old. She requires emergency medical evaluation. Follow local pediatric consent and medical control protocols. **3. Why request ALS if the pt is alert with relatively stable vitals?** A: Overdose can cause delayed complications requiring advanced monitoring and interventions.

	4. Why ask directly about suicidal intent? A: To determine whether ingestion was intentional, assess ongoing self-harm risk, and guide patient safety and psychiatric evaluation.

Psychiatric Emergency: 28 YO Male Experiencing Acute Agitation, Paranoia, and Hallucinations

Scenario Set Up	Equipment: - Stethoscope - Blood pressure cuff - Pulse oximeter - Thermometer - Glucometer - PPE - EMS radio **PROCTOR:** The patient is a 28-year-old male found in his apartment by his roommate. The roommate has called 911 because the patient has been pacing, shouting at unseen people, and claiming that someone is trying to kill him. The patient is frightened, suspicious of strangers, and reluctant to let anyone approach him. He initially refuses assessment because he believes the EMTs may be part of the threat. He is not initially violent but becomes more agitated if the EMTs crowd him, touch him without warning, argue with his beliefs, or make sudden movements. The roommate is nearby and can provide medical history if asked. The roommate reports that the patient has barely slept for two nights and has stopped taking a prescribed psychiatric medication several days ago.
Dispatch	Respond to a private residence for a 28-year-old male experiencing a psychiatric emergency. Caller reports that the patient is yelling at people who are not there, pacing, and acting paranoid.
Scene Size Up	The roommate is in the hallway, and the patient is pacing in the living room. Number of patients: 1 Nature of illness: Behavioral or psychiatric emergency with possible medication nonadherence. Environmental concerns: Cluttered coffee table and scattered belongings. No hazards identified. Life-threatening injuries: No obvious trauma, bleeding, or other injuries

Pertinent Primary Assessment Findings	AVPU - Alert A&O X 2 Oriented to person and place, but unsure of date and event A - Patent. The patient is speaking in full sentences. B - Spontaneous and unlabored, with a regular rate and adequate chest rise. No cyanosis or obvious respiratory distress. C - Radial pulse present, regular, and mildly tachycardic. Skin is warm and slightly diaphoretic. Capillary refill is less than two seconds.
Pertinent Secondary Assessment Findings	S: Agitated, paranoid, hearing voices, frightened, and poor sleep for two nights. Denies physical complaints. A: No known medication allergies; other allergies uncertain. M: Prescribed antipsychotic; missed doses for about four days. Medication and dose unknown. P: Schizophrenia; no known diabetes or seizure disorder. L: Small meal last evening; water this morning. E:: Increasing paranoia and agitation, shouting at unseen people and accusing neighbors of harm. Roommate called 911 due to escalating behavior. O: Symptoms worsened over the past 24–48 hours. P: Poor sleep and stressful interactions appear to worsen the symptoms. Calm conversation helps him feel safer. Q: Patient describes feeling watched and hearing threatening voices. R: Not applicable to the primary complaint. S: Patient rates his fear as 9/10. T: Increasing symptoms over the past two days.

Vitals	 Initial ● **BP:** 146/88 mmHg ● **HR:** 112 beats/min, regular ● **RR:** 20 breaths/min, regular and unlabored ● **BGL:** 104 mg/dL ● **SpO_2:** 98% on room air ● **Temperature:** 99.0°F ● **Skin:** Warm, slightly diaphoretic ● **Pain:** 0/10 ● **GCS:** 15 After Reassessment ● **BP:** 136/82 mmHg ● **HR:** 98 beats/min ● **RR:** 18 breaths/min ● **BGL:** N/A ● **SpO_2:** 98% on room air ● **Temperature:** 99.0°F
Treatments	 Necessary Interventions 1. Use therapeutic communication. Introduce yourself, speak slowly and calmly, use simple sentences, maintain a nonthreatening posture, and provide personal space. 2. Assess ABCs and mental status. Identify immediate threats and repeat assessments if the patient's condition changes. 3. Check vital signs and blood glucose. Evaluate for potentially reversible medical causes of altered behavior. 4. Obtain SAMPLE history and collateral information. Ask the roommate about medications, substance use, recent illness, sleep, and changes from baseline. 5. Assess suicide and violence risk. Ask direct, nonjudgmental questions and communicate any identified risk to the receiving team. 6. Arrange appropriate transport. Explain the purpose of evaluation and encourage voluntary cooperation. Follow local protocols and applicable law if the patient refuses care or appears to present an immediate danger to self or others.

Key Points	- Identify psychiatric crises while considering possible medical causes and immediate life threats. - Maintain scene safety and use therapeutic communication and de-escalation. - Gather SAMPLE history, obtain vital signs and blood glucose as appropriate, and ask about suicide, self-harm, or harm to others.
Bonus Questions	**Why should EMTs check blood glucose in a patient with unusual behavior?** Hypoglycemia can cause confusion, agitation, altered mental status, and behavior that resembles a psychiatric crisis. **What are some other medical causes of acute agitation or altered behavior?** Hypoxia, intoxication, drug overdose or withdrawal, head injury, infection, stroke, seizures, metabolic disturbances, and other neurologic or medical conditions.

Psych Emergency: 41 YO Female, Acute Stress Reaction After Witnessing a Fatal Crash

Scenario Set Up	**Scenario Set Up** Equipment needed: BSI/PPE, BP cuff, pulse ox, glucometer, stethoscope, penlight, O2 w/ NC & NRB, gurney, blanket, water, radio, AED PROCTOR: You are Maria, 41. About 20 min ago, while you were driving home from work, the car in front of you was hit head-on by a vehicle that crossed the center line. You stopped and tried to help the driver while waiting for 911, but he died before help arrived. You are uninjured, and your car was not involved. You're sitting on the curb about 50 ft from the wreck, rocking, hands shaking, breathing fast. You keep staring at the crash and saying, "I couldn't do anything. I should have done something." A police officer called EMS because you "don't look right." You feel a strong urge to keep looking at the crash. Your hands and lips are tingling, your chest is tight, and you're scared you're having a heart attack. ● If moved out of view of the crash (into the ambulance or behind a vehicle): calm down noticeably. ● If told "calm down," "there's nothing you could have done," or "it's not a big deal": get more upset ("You weren't there! You don't understand!") and breathe faster. ● If they acknowledge your feelings and coach slow breathing: breathing slows within a few minutes and the tingling fades. ● If asked about chest pain: it's tightness that started when you began breathing fast, doesn't radiate, and improves as breathing slows. ● If asked about self-harm: "No, nothing like that. I just keep seeing it." ● Sirens: fine if warned first. If a siren starts without warning, startle and breathe faster briefly. ● At first you want to drive yourself home. Agree not to if they explain why and offer to call someone. You want your husband called.
Dispatch	Respond to a two-vehicle head-on MVC with multiple units already on scene. PD is requesting a separate unit for a 41 YOF bystander not involved in the crash. She is conscious and breathing but appears anxious and SOB.
Scene Size Up	● Active MVC with PD controlling traffic. Fire and other EMS units are working the crash, so stay clear of the wreck and traffic. Pt is on the curb about 50 ft from the crash with a clear view of it, rocking, crying, and breathing rapidly. 1 pt assigned to your crew. NOI: acute stress reaction after witnessing a traumatic event; r/o cardiac and other medical causes. No

	visible life threats. The blood on her hands and clothes is from the other pt; she has no injuries of her own.
Pertinent Primary Assessment Findings	● AVPU: Alert. A&O x4 (person, place, time, event), but distracted and keeps looking at the crash. ● A - patent; speaking in short phrases between rapid breaths ● B - rapid, deep, regular, RR 32; breath sounds clear and equal bilaterally; no accessory muscle use ● C - radial pulse rapid, strong, regular; skin pale, warm, diaphoretic; CRT <2 sec; no bleeding found
Pertinent Secondary Assessment Findings	Symptoms: chest tightness, tingling in both hands and around the lips, lightheadedness, shaking, racing heart. Slight carpopedal spasm that resolves as breathing slows. Repeated thoughts: "I couldn't do anything." "I keep seeing his face." Denies injury, head strike, LOC, and thoughts of self-harm. OPQRST (chest tightness): ● O - started a few minutes after the driver died, as she began breathing fast ● P - worse when she looks at the crash, better with slow breathing ● Q - tight, "like a band" ● R - center of chest, no radiation ● S - 5/10 ● T - about 15 min, improving with coaching SAMPLE: ● A - NKA ● M - daily multivitamin only ● P - no cardiac hx; father had an MI at 58; no psych hx or panic attacks ● L - lunch about 5 hrs ago, coffee in the afternoon ● E - driving home, witnessed the head-on collision, stopped and tried to help the driver, who died before EMS arrived; began hyperventilating afterward Physical exam: no trauma, lungs clear, PERRL, no neuro deficits once breathing normalizes.

Vitals	Initial (on curb, in view of crash): BP 148/92, HR 122, RR 32, BGL 110, SpO2 100% RA, T 98.6°F, Pain 5/10 After moving out of view, breathing coaching, and validation: BP 132/84, HR 96, RR 18, SpO2 99% RA, Pain 1/10, tingling resolved
Treatments	Necessary: ● Ensure scene safety and stay clear of traffic and the crash. ● Move her out of view of the crash (into the ambulance or behind a vehicle). ● Introduce yourselves calmly, use her name, and sit at her level. ● Complete the primary assessment; don't assume anxiety without checking. ● Use OPQRST for the chest tightness and r/o cardiac causes. Follow the local chest pain protocol given her age and family hx. ● Coach slow, steady breathing matched yours. Do NOT use a paper bag. ● Give O2 only if indicated by assessment or protocol. ● Get full vitals, BGL, and SAMPLE. ● Check for her own injuries, since she has blood on her clothes. ● Ask directly about self-harm. ● Reassess vitals and symptoms. ● Offer ED transport, especially given the chest pain. If she refuses, follow local refusal protocol and assess capacity. Additional: ● Validate her feelings; avoid saying "calm down" or "there was nothing you could have done." ● Don't share details of the other pt's condition or injuries. ● Let her wash her hands or give her wipes and a blanket. ● Warn her before sirens or loud noises. ● Offer to call her husband or another support person. ● Don't let her drive home. ● Provide crisis or counseling resources per local protocol.
Key Points	**Key Points** ● Bystanders and witnesses can be pts too; witnessing a death or traumatic event can cause an acute stress reaction.

	● Removing the pt from the trigger (the view of the crash) is one of the fastest interventions. ● Hyperventilation can cause chest tightness, tingling, and hand cramping, but you must still r/o cardiac and medical causes, especially with a family hx. ● Never use a paper bag: it can cause hypoxia and hide other conditions. ● Validate feelings; dismissing them makes things worse. ● Don't tell her she could or couldn't have saved him; acknowledge that she tried to help. ● Screen for self-harm even when it seems unlikely. ● A pt in an acute stress reaction should not drive; arrange a safe ride home. ● Psychological first aid: safety, calm, connection, and a plan for follow-up.
Bonus Questions	**Bonus Questions** ● Why shouldn't EMTs use a paper bag for hyperventilation? (A: It can lower O2 and cause hypoxia, and it can mask serious causes of rapid breathing like MI, PE, asthma, or DKA.) ● What causes the tingling and hand cramping? (A: Fast breathing drops blood CO2, causing respiratory alkalosis. That lowers ionized calcium, leading to tingling and spasms like carpopedal spasm.) ● What medical conditions can look like a panic attack? (A: MI, arrhythmia, PE, asthma attack, hypoglycemia, hyperthyroidism, and drug use or withdrawal.) ● What should EMTs say if she asks whether the driver could have been saved? (A: Don't speculate or share details. Acknowledge that she helped: "You stayed with him and tried to help. That matters.")